

● Pathology ● Sonography ● Digital X-Ray ● Colour Doppler ● ECG ● Echocardiography ● TMT ●

Regn No. : 10304250079
Name : Mr. Jugal Kishore
Sex/Age : Male / 33 Years
Ref. By : DR. SELF
Client Name : ECHIKITSALAYA (UNO.CARE)

Registered On : 03-Apr-2025 11:28
Sample Collected On : 03-Apr-2025 11:37
Sample Reported On : 03-Apr-2025 14:38
Regn Centre : Shivani Diagnostic Centre
Sample Type : EDTA Blood

Payment Status : Paid

HAEMATOLOGY

Parameters	Result	Unit	Biological Reference Interval
Complete Blood Counts (CBC - 5 Part)			
Haemoglobin (Hb)	14.0	gms/dl	13.5 - 17.5
RBC Count	5.07	milli./cu.mm	4.5 - 5.9
Packed Cell Volume (HCT)	43.9	%	37.0 - 53.0
Mean Corpuscular Volume (MCV)	86.5	fL	80.0 - 100.0
Mean Corpuscular Hemoglobin (MCH)	27.7	pg	26.0 - 34.0
Mean Corpuscular Hb Conc (MCHC)	31.9	gms/dl	32.0 - 36.0
RDW CV	13.5	%	11.0 - 16.0
RDW SD	41.9	fL	35.0 - 56.0
White Blood Cells and Differential Count			
Total WBC Count	5810	/cumm	4500 - 11000
Neutrophils	55	%	35 - 75
Lymphocytes	39	%	24 - 44
Monocytes	05	%	2 - 12
Eosinophils	01	%	0 - 6
Basophils	00	%	0 - 1
Absolute Count			
Absolute Neutrophil Count	3196	/cumm	1575 - 8250
Absolute Lymphocyte Count	2266	/cumm	1080 - 4840
Absolute Monocyte Count	291	/cumm	90 - 1320
Absolute Eosinophil Count	58	/cumm	0 - 660
Immature Granulocyte (IMG)	0.1	%	0.0 - 1.0
Neutrophil-to-lymphocyte ratio, NLR	1.4	%	0.78 - 3.35
Platelets			
Platelet Count	2.26	Lakh/cumm	1.5 - 4.5
Mean Platelet Volume (MPV)	9.5	fL	6.5 - 12.0
Platelet Distribution Width (PDW)	16.2		15.0 - 17.0

Performed on:- "Mindray BC-6200" Fully Automated 5 Part Hematology Analyzer.

Methods:- Hemoglobin by Cyanide free method. Total RBC count, platelets count by electrical Impedance method. PCV, MCV, MCH, MCHC: calculated, WBC Parameters by SF CUBE cell technology method/microscopy. Reference:- Nathan DG, and Oski, FA (1981) Hematology of Infancy and Childhood, ed 2, WB Saunders, pp1552. Dallman PR (1977) pediatrics, 16th edition, Appleton-Century-Crofts, p 1111 pediatrics Normal Range (1995) Children's Hospital of Buffalo, Coulter Viewpoint, No.17. p 8 Pediatrics Normal Range study (1995) Children's Hospital, Minneapolis and St. PAUL.

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MC-2597



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Lab Director, Pathologist

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Haematology

ERYTHROCYTE SEDIMENTATION RATE

Parameters	Result	Unit	Biological Reference Interval
Corrected ESR <i>Westergen's Method</i>	22	mm/hr	0 - 10

Reference :- Dacie and Lewis - Practical Haematology

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Sample Type : Serum

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BIOCHEMISTRY

CREATININE WITH EGFR

Parameters	Result	Unit	Biological Reference Interval
Creatinine Serum <i>Enzymatic Method</i>	0.83	mg/dL	0.5 - 1.5
eGFR <i>Method: Calculated</i>	115.6	mL/Min/1.73M	>60

Interpretation :

- Clinical Significance :-The normal serum creatinine reference interval dose not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone.Thus,it is recommended for clinical laboratories to routinely estimate glomerular filtration rate(eGFR), a "gold standard" measurement for assessment of renal function and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible.It cannot be measured easily in clinical practice,instead,GFR is estimated from equations using serum creatinine, age , race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Applction of CKD- EPI equation together with the other diagnostic tools in renal medicine will further imporve the detection and mangement of patients with CKD.

Reference :

Levey AS, Stevens LA, Schmid CH,Zhang YL,Castro AF,3rd,Feldman HI, et al. A new equation to estimate glomerular filtration rate, Ann Intern Med. 2009;150(9):604-12. Please correlate with clinical conditions.

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Sample Type : Serum, Fluoride Plasma

Payment Status : Paid

BIOCHEMISTRY

Parameters	Result	Unit	Biological Reference Interval
SGPT Method : UV assay according to IFCC	12	U/L	0 - 45
SGOT Method : UV assay according to IFCC	16	U/L	0 - 35
Random Blood Sugar			
RBS Method : GOD-POD	100	mg/dL	Normal : <140 Diabetic : =200 + Symptoms

Treatment Goals for Adults with Diabetes-

- 1) Pre-Prandial plasma glucose (FBS) 80-130 mg/dl
- 2) Post-Prandial plasma glucose (PPBS) <180 mg/dl
- 3) HbA1c <7.0%
- 4) Blood pressure <140/90 mmHg

*Source: Adapted American Diabetes Association: Diabetes Care 41(suppl 1):s86, 2018.

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Sample Type : Serum

Payment Status : Paid

SEROLOGY

VDRL

Parameters	Result	Unit	Biological Reference Interval
VDRL <i>Flocculation</i>	Non-Reactive		Non-Reactive

Interpretation :

The test will only indicate the presence of Treponema pallidum (TP) antibodies in the specimen and should not be used as the sole criteria for the diagnosis of TP infection. All results must be interpreted together with clinical information available to the physician. A negative result does not at any time preclude the possibility of TP infection.

LIMITATIONS

1. RPR (Rapid Plasma Reagin) test kit is a rapid screening test. A positive result must be reconfirmed with tests employing specific treponemal antigen e.g. TPFA, FTA-ABS etc. as well as other clinical findings.
2. Biological false positive reactions (BFP) can be attributed to Malaria, Hepatitis, Mumps, Leprosy, Infectious Mononucleosis, Rheumatoid Arthritis and Collagen disease. When BFPs do occur, they may be of low titer (< 1:8 dilution).
3. Presence of dust, high heat, poor light and the drying of the reaction mixture before being read also affect results (particularly when the test is performed in field conditions).

Remark :

Biological False positive reaction occurs frequently and has been attributed to a variety of acute and chronic condition.in absence of supporting clinical, historical or epidemiological evidence, reactive result must be confirmed with more specific treponemal tests like TPFA.

References:

- (I) E.H. Lennette, A. Balows, W.J.Hausler Jr., H.J. Shadomy, Manual of Clinical Microbiology: 4th ed. (Washington), 1985,485-489.
- (II) A.E. Singh and Romanowski, Syphilis: Review with emphasis on Clinical, Epidemiologic and some Biologic Features, Clinical Microbiology Reviews (1999)15:187-209.

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[Signature]

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Sample Type : Urine

Payment Status : Paid

URINE ROUTINE EXAMINATION

Parameters	Result	Unit	Biological Reference Interval
Physical Examination			
Quantity	20 ml	ml	
Colour	Pale Yellow		Pale Yellow
Appearance	Clear		Clear
pH	6.0		5.0 - 8.5
Specific Gravity	1.015		1.000 - 1.030
Chemical Examination			
Protein	Nil		Nil
Glucose	Nil		Nil
Ketone	Nil		Nil
Bilirubin	Absent		Absent
Blood	Nil		Nil
Urobilinogen	Normal		Normal
Nitrite	Absent		Absent
Microscopic Examination			
Pus Cells	2-3	cells/HPF	0 - 5
Epithelial Cells	2-4	cells/HPF	0 - 5
Red Blood Cells	Nil	cells/HPF	0 - 2
Spermatozoa	Absent		Absent
Casts	Absent		Absent
Crystals	Absent		Absent
Yeast Cell	Absent		Absent
Bacteria	Absent		Absent

Method: Urinalysis-Multiple Reagent Strip Method & Microscopy done after centrifugation of 2500 rpm for 15 minutes. Glucose oxidase-peroxidase, protein - Tetrabromophenol blue, Ketone-Sodium nitroprusside, Blood-Peroxidase activity of hemoglobin, Bilirubin-Diazotized dichloroaniline, Urobilinogen - Ehrlich Method. Please look for drug history (SGLT-2 inhibitors) in diabetic patient if urine sugar is irrational high. Glucose is recorded as Nil to Present +++++, Bilirubin is recorded as absent or present, Ketone is recorded Nil to Present +++++, Blood is recorded as Nil to Present +++++, Protein is recorded as Nil to Present +++++, Urobilinogen is recorded as Normal to Present +++++, Nitrite is recorded as Absent to Present.

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Sex/Age	: Male / 33 Years	Sample Reported on	: 03-Apr-2025 15:19	
Ref. By	: Dr. SELF	Regn Centre	: Shivani Diagnostic Centre	
Client Name	: Echikitsalaya (UNO.CARE)		: Paid	

X-RAY CHEST PA VIEW.

Trachea central in position.

Cardio thoracic ratio within normal limits.

Both lung fields are clear.

Both hilar markings are normal.

Both C.P. angles are clear.

Bony cage & soft tissue appear normal.

Kindly correlate clinically.

----- End Of Report -----

Dr. Kapil kumar Uikey
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